Monitazone 50mcg
actuation Nasal Spray

Mometasone Furoate 0.5mg per ml of suspension

List of Excipients
Microcrystalline Cellulose and Carboxymethylcellulose Sodium (Avicel), Glycerine, Citric Acid Hydrous, Sodium Citrate Dihydrate, Polysorbate 80, Benzalkonium Chloride, Purified Water

Dosage Form
Liquid Nasal Spray.

Product Description
White to off-white opaque suspension in a container fitted with a metered spray pump

Pharmacodynamics/ Pharmacokinetics
Pharmacodynamics
Mometasone furoate nasal spray has shown anti-inflammatory activity in both the early- and late- phase allergic responses. This has been demonstrated by decreases in histamine and eosinophils, neutrophils and epithelial cell adhesion proteins. Mometasone furoate nasal spray showed significant improvement when compared to placebo in the clinically relevant endpoints of congestion, nasal polyp size and loss of smell.

Mechanism of Action
Mometasone furoate is a topical glucocorticosteroid with local anti-inflammatory properties at doses that are not systemically active. It is likely that much of the mechanism for the anti-allergic and anti-inflammatory effects of mometasone furoate lies in its ability to inhibit the release of mediators of allergic reactions. Mometasone furoate significantly inhibits the release of leukotrienes from leucocytes of allergic patients. In cell culture, mometasone furoate demonstrated high potency in inhibition of synthesis and release of IL-1, IL-5, IL-6 and TNF alpha; it is also a potent inhibitor of leukotriene production. In addition, it is an extremely potent inhibitor of the production of the Th2 cytokines, IL-4 and IL-5, from human CD4+ T cells.

Pharmacokinetics
Absorption
Mometasone furoate, administered as an aqueous nasal spray, has a systemic bioavailability of <1% in plasma, using a sensitive assay with a lower quantitation limit of 0.25pg/ml.

Distribution
Not applicable as mometasone is poorly absorbed via the nasal route.

Biotransformation
The small amount that may be swallowed and absorbed undergoes extensive first-pass hepatic metabolism.

Elimination
Absorbed mometasone furoate is extensively metabolized and the metabolites are excreted in urine and bile.

Indication
Monitazone Nasal Spray is indicated for use in adults, adolescents and children between the ages of 3 and 11 years to treat the symptoms of seasonal or perennial rhinitis. In patients who have a history of moderate to severe symptoms of seasonal allergic rhinitis, prophylactic treatment with Monitazone Nasal Spray is recommended two to four weeks prior to the anticipated start of the pollen season. Monitazone Nasal Spray is also indicated for the treatment of nasal polyps in patients 18 years of age and older. Monitazone Nasal Spray is
indicated for the treatment of symptoms associated with acute rhinosinusitis in patients 12 years of age and older without signs or symptoms of severe bacterial infection.

Recommended Dose
After initial priming of the Monitazone Nasal Spray pump (10 actuations, until a uniform spray is observed), each actuation delivers approximately 100mg of mometasone furoate suspension, containing mometasone furoate monohydrate equivalent to 50 micrograms mometasone furoate. If the spray pump has not been used for 14 days or longer, it should be re-primed with 2 actuations until a uniform spray is observed before next use.

Shake container well before each use. Seasonal allergic or perennial rhinitis: Adults (including geriatric patients) and adolescents: The usual recommended dose for prophylaxis and treatment is two sprays (50 micrograms/spray) in each nostril once daily (total dose 200 micrograms). Once symptoms are controlled, dose reduction to one spray in each nostril (total dose 100 micrograms) may be effective for maintenance. If symptoms are inadequately controlled, the dose may be increased to a maximum daily dose of four sprays in each nostril once daily (total dose 400 micrograms).

Dose reduction is recommended following control of symptoms. Clinically significant onset of action occurs as early as 12 hours after the first dose. Children between the ages of 3 and 11 years: The usual recommended
dose is one spray (50 micrograms/spray) in each nostril once daily (total dose 100 micrograms). Administration to young children should be aided by an adult. Nasal polyposis: Adults (including geriatric patients) and adolescents
18 years of age and older: The usual recommended dose for polyposis is two sprays (50 micrograms/spray) in each nostril twice daily (total daily dose of 400mcg).

Once symptoms are adequately controlled, dose reduction to two sprays in each nostril once daily (total daily dose 200mcg) is recommended. Acute Rhinosinusitis: Adults (including geriatric patients) and adolescents 12 years of age or older: The usual recommended dose for acute rhinosinusitis is two actuations (50 micrograms / actuation) in each nostril twice daily (total daily dose of 400 micrograms). If no improvement is seen after 15 days of twice daily administration, alternative therapies should be considered. If symptoms worsen during treatment, the patient should be advised to consult their physician.

How to Use Monitazone Nasal Spray

Figure1

Figure2

Figure3

(a) Shake the bottle gently, remove the dust cap and safety pin. (Figure 1)
(b) When you use Monitazone for the first time, prime the pump by pressing downward on the nasal applicator using your index finger and middle finger while holding the base of the bottle with your thumb. Press down and release the pump 5 or 10 times until a fine spray appears. If unused for more than 2 weeks, prime the pump again 2 times until a fine spray appears. (Figure 2)
(c) Gently blow your nose to clear the nostrils. Close one nostril using your finger and put the nozzle into the other nostril.
(d) Tilt your head forward slightly, keeping the bottle upright. (Figure 3)
(e) Start to breathe in gently or slowly through your nose and whilst you are breathing in, squirt a spray of fine mist into your nose by pressing down ONCE with your fingers.
(f) Breathe out through your mouth. Repeat step (e) to inhale a second spray in the same nostril if applicable.
(g) Remove the nozzle from this nostril and breathe out through the mouth.
(h) Repeat steps (c) to (g) for the other nostril. Do not use more than directed number of times.

After using the spray, wipe the nozzle carefully with a clean handkerchief or tissue and replace the dust cap.

Route of Administration
Nasal

Contraindication
Hypersensitivity to the active substance, mometasone furoate, or to any of the excipients. Monitazone Nasal Spray should not be used in the presence of untreated localized infection involving the nasal mucosa such as herpes simplex. Because of the inhibitory effect of corticosteroid on would healing, patients who have experienced recent nasal surgery or trauma should not use a nasal corticosteroid until healing has occurred.

Warnings & Precautions
Immunosuppression
Monitazone Nasal Spray should be used with caution, if at all, in patients with active or quiescent tuberculous infections of the respiratory tract, or in untreated fungal, bacterial, or systemic viral infections. Patients receiving corticosteroids who are potentially immunosuppressed should be warned of the risk of exposure to certain infections (e.g., chickenpox, measles) and of the importance of obtaining medical advice if such exposure occurs.

Local Nasal Effects
Persistence of nasopharyngeal irritation may be an indication for discontinuing Monitazone Nasal Spray. Monitazone Nasal Spray is not recommended in case of nasal septum perforation. Monitazone Nasal Spray contains benzalkonium chloride which may cause nasal irritation.

Systemic Effects of Corticosteroids

Systemic effects of nasal corticosteroids may occur, particularly at high doses prescribed for prolonged periods. These effects are much less likely to occur than with oral corticosteroids and may vary in individual patients and between different corticosteroid preparations. Potential systemic effects may include Cushing's syndrome, Cushingoid features, adrenal suppression, growth retardation in children and adolescents, cataract, glaucoma and more rarely, a range of psychological or behavioural effects including psychomotor hyperactivity, sleep disorders, anxiety, depression or aggression (particularly in children).
Following the use of intranasal corticosteroids, instances of increased intraocular pressure have been reported. Patients who are transferred from long-term administration of systemically active corticosteroids to Monitazone Nasal Spray require careful attention. Systemic corticosteroid withdrawal in such patients may result in adrenal
insufficiency for a number of months until recovery of HPA axis function. If these patients exhibit signs and symptoms of adrenal insufficiency or symptoms of withdrawal (e.g., joint and/or muscular pain, lassitude, and depression initially) despite relief from nasal symptoms, systemic corticosteroid administration should be resumed and other modes of therapy and appropriate measures instituted. Such transfer may also unmask pre-existing allergic conditions, such as allergic conjunctivitis and eczema, previously suppressed by systemic corticosteroid therapy.

Treatment with higher than recommended doses may result in clinically significant adrenal suppression. If there is evidence for higher than recommended doses being used, then additional systemic corticosteroid cover should
be considered during periods of stress or elective surgery.

Nasal Polyps
The safety and efficacy of Monitazone Nasal Spray has not been studied for use in the treatment of unilateral polyps, polyps associated with cystic fibrosis, or polyps that completely obstruct the nasal cavities. Unilateral polyps that are unusual or irregular in appearance, especially if ulcerating or bleeding, should be further evaluated.

Effects on Growth in Paediatric Population
It is recommended that the height of children receiving prolonged treatment with nasal corticosteroids is regularly monitored. If growth is slowed, therapy should be reviewed with the aim of reducing the dose of nasal corticosteroid if possible, to the lowest dose at which effective control of symptoms is maintained. In addition, consideration should be given to referring the patient to a paediatric specialist.

Non-nasal Symptoms
Although Monitazone Nasal Spray will control the nasal symptoms in most patients, the concomitant use of appropriate additional therapy may provide additional relief of other symptoms, particularly ocular symptoms.

Interactions with Other Medicaments
See warning & precautions for use with systemic corticosteroids.

No interactions with loratadine.

Co-treatment with CYP3A inhibitors, including cobicistat-containing products, is expected to increase the risk of systemic side-effects. The combination should be avoided unless the benefit outweighs the increased risk of systemic corticosteroid side-effects, in which case patients should be monitored for systemic corticosteroid side- effects.

Pregnancy and Lactation
Pregnancy
There are no or limited amount of data from the use of mometasone furoate in pregnant women. Studies in animals have shown reproductive toxicity. As with other nasal corticosteroid preparations, Monitazone Nasal Spray should not be used in pregnancy unless the potential benefit to the mother justifies any potential risk to the
mother, foetus or infant. Infants born of mothers who received corticosteroids during pregnancy should be observed carefully for hypoadrenalism.

Lactation
It is unknown whether mometasone furoate is excreted in human milk. As with other nasal corticosteroid preparations, a decision must be made whether to discontinue breast-feeding or to discontinue/abstain from Monitazone Nasal Spray therapy taking into account the benefit of breast feeding for the child and the benefit of therapy for the woman.

Fertility
There are no clinical data concerning the effect of mometasone furoate on fertility. Animal studies have shown reproductive toxicity, but no effects on fertility.

Side Effects
Summary of the safety profile
Epistaxis was generally self-limiting and mild in severity. In patients treated for nasal polyposis, the overall incidence of adverse events was similar to that observed for patients with allergic rhinitis. Systemic effects of nasal corticosteroids may occur, particularly when prescribed at high doses for prolonged periods.

Tabulated list of adverse reactions
Treatment related adverse reactions (≥1%) reported in patients with allergic rhinitis or nasal polyposis and post- marketing regardless of indication are presented in Table 1.

Table 1: Treatment-related adverse reactions reported by system organ class and frequency

	Very Common	Common	Not Known
Infections and infestations		Pharyngitis, upper respiratory tract infection†	
Immune system disorders			Hypersensitivity including anaphylactic reactions, angioedema, bronchospasm and dyspnoea
Nervous system disorders		Headache	
Eye disorders			Glaucoma, increased intraocular pressure, cataracts
Respiratory, thoracic and mediastinal disorders	Epistaxis*	Epistaxis, nasal burning, nasal irritation, nasal ulceration	Nasal septum perforation
Gastrointestinal disorders		Throat irritation*	Disturbances of taste and smell

* Recorded for twice daily dosing for nasal polyposis.
† Recorded at uncommon frequency for twice daily dosing for nasal polyposis.

Paediatric Population
In the paediatric population, adverse events of epistaxis, headache, nasal irritation and sneezing was reported.

Symptoms and Treatment of Overdose
Symptoms
Inhalation or oral administration of excessive doses of corticosteroids may lead to suppression of HPA axis function.

Management
Because the systemic bioavailability of Monitazone Nasal Spray is <1%, overdose is unlikely to require any therapy other than observation, followed by initiation of the appropriate prescribed dosage.

Storage Condition
Store below 30°C in tight container.

Shelf Life
24 months

Therapeutic Code
R01AD09

Pack Size
140 sprays

Product Owner
ILDONG PHARMACEUTICAL CO., LTD.

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Product Registration Holder
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설명서	모니타존 나잘스프레이(말레이시아)	
학술담당	디자인담당	품질담당
강예지 과장	오인호 차장	윤미숙 차장
	변경 전	현재
화판번호	(MY)1235995-1180115	(MY)1235995-1211214
사이즈(mm)	160 x 230	변동없음
지질/평량	모조지 70g	변동없음
코팅	무코팅	변동없음
변경 내역	화판번호, Product Description	
비고		
인쇄소		

색상	
	K
후가공	
별색판 / 중복인쇄 확인할 것	